

Criteria	Yes	No
Diagnosis Discrepancy		
Primary Tumor Site Discrepancy		
HIPAA Discrepancy		
Prior Malignancy History		
Dual/Synchronous Primary Noted		
Case Is (Circle):	QUALIFIED	DISQUALIFIED
Reviewer Initials	AKG	
Date Reviewed	5/13/11	

Final Pathologic Diagnosis:

A. Right neck, lymph nodes, level 2-4, dissection:
Metastatic papillary thyroid carcinoma in twelve out of twenty lymph nodes (12/20).

B. Thyroid gland, total thyroidectomy:
Papillary thyroid carcinoma.

Procedure: Total thyroidectomy with right neck
dissection and left contents neck mass (part C)

Specimen integrity: Intact

Specimen size:

Right lobe: 4.0 x 3.5 x 1.8 cm

Left lobe: 3.6 x 3.5 x 1.6 cm

Isthmus # pyramidal lobe: N/A

Central compartment: N/A

Right neck dissection: 13.0 x 4.8 x 1.8 cm

Left neck dissection (part C): 0.6 x 0.3 x 0.1 cm

Tumor focality: Unifocal

1CD-0-3

carcinoma, papillary thyroid
826013

Site: thyroid, Nos C73.9

hw
2/14/11

Dominant tumor

Tumor laterality: Right lobe

Tumor size: 2.5 cm in greatest dimension

Histologic type: Papillary thyroid carcinoma

Necrosis: Absent

Mitotic figures: 1 per 10 high-power fields

Margins: Uninvolved by carcinoma (the tumor is <1 mm
from resected surgical margin)

Tumor capsule: Partially encapsulated

Tumor capsular invasion: Present

Extent: Widely invasive

Lymph-vascular invasion: Present

Extent: Less than 4 vessels

Extrathyroidal extension: Focally present

Extent: Focal

Second tumor: N/A

Pathologic staging:

TNM descriptors: None known

Primary tumor: pT2

Regional lymph nodes: pNb

Number examined: 23

Number involved: 14 (12 out of 20 lymph

nodes are positive in part A; 2 out of 2 lymph nodes are positive in part B; 1

UUID:009D16C2-2C9C-4955-90B4-A49F54033C74
TCGA-GE-A2C6-01A-PR

Redacted



lymph node is negative in part C)
Distant metastasis: N/A

Additional pathologic findings: Hashimoto thyroiditis

C. Left content neck mass (FS), resection:
One lymph node, negative for metastatic tumor (0/1).
Frozen section diagnosis is confirmed.

The examination of this case material and the preparation of this report were performed by the staff pathologist.

Intraoperative Consult Diagnosis:

FSC: Left central neck mass:
Lymph node negative for malignancy.
6 mins.

Gross Description:

Received are three formalin filled containers each labeled with patient's name.

A is received and additionally labeled "A, right neck Level II-IV dissection, single stitch at Level II, double stitch at Level IV" and consists of a 13.0 x 4.8 x 1.8 cm purple-tan irregular soft tissue with a single black stitch denoting the Level II area and a double black stitch denoting Level IV. Palpation through the specimen reveals multiple firm nodularities. Sectioning reveals eight lymph nodes in the station two area the largest of which has a greatest dimension of 3.4 cm and has apparent calcifications within. There are six probable station four lymph nodes the largest of which have a greatest dimension of 3.5 cm. These nodes are submitted in its entirety as follows:

- A1 one whole lymph node, entirely;
- A2 one whole lymph node, station two, entirely;
- A3 two whole lymph nodes, entirely;
- A4 two whole lymph nodes, bisected, all halves submitted;
- A5 two whole lymph nodes, each bisected, all halves submitted;
- A6,7 one whole lymph node, step-sectioned, entirely;
- A8-12 largest station two lymph node, step-sectioned, entirely;
- A13 two probable lymph nodes, station four, entirely;
- A14 two probable lymph nodes, station four, entirely;
- A15 two whole lymph nodes, each bisected, all halves submitted;
- A16 one whole lymph node, bisected, each half submitted;
- A17-19 one whole lymph node, station four, step-sectioned, entirely;
- A20-23 largest station four lymph node, step-sectioned, entirely.

B is received and additionally labeled "total thyroidectomy; central neck dissection, single left upper pole, double at right lower pole" and consists of a total thyroid specimen 5.5 x 6.0 x 1.5 cm. The thyroid is received with a

single stitch denoting the left upper pole that is also inked blue and a double stitch denoting the right lower pole with the overlying surface inked black. The surface is diffusely nodular and multiple staples are contained within the surface. A single incision has been made through the _____ are completed for each side so that the entire specimen is inked as follows: right black, left blue and the specimen is sectioned to reveal discloses a cut surface of the thyroid that is remarkable for a poorly demarcated mass in the right lobe 2.2 x 2.5 x 1.4 cm. No grossly identifiable lesion is apparent in the left lobe. Sectioning through the specimen reveals that the tissue in the inferior of the specimen is comprised of lymph nodes. Many of these nodes appear to matted and confluent and accurate count is not apparent but there are some that are able to separate from the mass in estimation of at least six nodes present are made most of which appear grossly involved. Representative sections are submitted as following:

- B1,2 right lobe to include mass in relationship to inked surgical margin;
- B3,4 apparent uninvolved left lobe;
- B5 apparent isthmus;
- B6 representative node;
- B7 representative node.

C is received and additionally labeled "left central neck mass and FSC+0" and consists of 0.6 x 0.3 x 0.1 cm pink-tan soft tissue received fresh for frozen section diagnosis. The specimen is submitted in its entirety in cassette C1.

Microscopic Description:

The final diagnosis of each specimen incorporates the microscopic examination findings.

END OF REPORT
